

Response to HHS Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

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I. Introduction

ShiftOS is a healthcare technology company developing AI-powered workforce operations software for healthcare organizations. Our product, Holly, is an AI scheduling agent that automates the complex, time-intensive work of managing clinical staff schedules; including shift assignments, credential compliance, call-offs, shift swaps, and float pool optimization.

My background informs our approach: I served as a Hospital Corpsman in the U.S. Navy, worked at the National Cancer Institute on the Cancer Moonshot initiative under the NCI Associate Director, and led data delivery operations at IBM Watson Health working with MarketScan claims data. My co-founder spent nearly six years as a pharmacy technician at CVS while completing school, witnessing firsthand the administrative burden that drives clinician burnout.

We are currently deploying Holly with enterprise health systems and have established integration partnerships with major workforce management platforms including Workday. Our perspective on AI adoption barriers comes from direct experience navigating procurement, IT integration, and clinical operations stakeholder alignment at large healthcare organizations.

We appreciate the opportunity to provide input on this RFI and offer the following responses to selected questions.

II. Responses to Specific Questions

Question 1: What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

The barriers we encounter most frequently are not technical — they are structural and procedural:

1. Fragmented procurement and IT governance.

Healthcare organizations often lack clear pathways for evaluating and adopting operational AI tools. Clinical AI (diagnostic, imaging) has established FDA frameworks and clinical validation processes. Operational AI: scheduling, staffing, administrative automation, falls outside these frameworks but still faces lengthy procurement cycles (often 12-18 months) because organizations default to enterprise IT evaluation processes designed for EHRs and clinical systems. There is no proportionate, streamlined pathway for lower-risk operational AI.

2. Integration complexity with legacy workforce systems.

Health systems operate fragmented technology stacks: one system for scheduling, another for time and attendance, another for credentialing, another for payroll. These systems rarely interoperate well. AI tools that could automate scheduling decisions require read/write access to multiple systems simultaneously: scheduling, credential databases, union rules engines, payroll; and each integration requires separate security reviews, BAAs, and technical work. The absence of standard APIs or interoperability requirements for workforce management systems creates friction that delays deployment by months.

3. Credentialing and compliance data silos.

Real-time scheduling decisions require real-time visibility into staff credentials, licenses, certifications, and competencies. This data is typically locked in credentialing software that does not expose it programmatically, forcing manual verification or batch exports. This limits AI's ability to make compliant staffing decisions autonomously.

4. Risk aversion without clear accountability frameworks.

Healthcare leaders are uncertain about liability when AI systems make or recommend staffing decisions. Who is accountable if an AI schedules an under-credentialed provider? The lack of clear guidance from HHS on accountability for non-medical-device AI creates hesitation among compliance officers and legal teams, slowing adoption even when operational leaders are enthusiastic.

Question 2: What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why?

1. Extend interoperability mandates to workforce management systems.

The 21st Century Cures Act and ONC's information blocking rules have improved data liquidity for clinical information. Similar interoperability requirements should apply to workforce management, credentialing, and time/attendance systems. Health systems should be able to access their own workforce data. Schedules, credentials, availability — through standardized APIs without vendor lock-in or prohibitive integration fees.

Recommendation: HHS should consider whether workforce management data constitutes a category of information that, when siloed, impedes the effective delivery of care, and evaluate extending information blocking provisions or publishing voluntary interoperability standards for this data category.

2. Clarify accountability frameworks for operational AI.

HHS should publish guidance distinguishing operational AI (scheduling, staffing, administrative automation) from clinical decision support and medical devices. This guidance should address liability allocation, documentation requirements, and human oversight expectations for AI systems that make non-clinical operational decisions.

This clarity would reduce legal uncertainty that currently delays procurement and allow health systems to adopt operational AI with appropriate, but not excessive, governance.

3. Recognize administrative burden reduction as a quality and safety priority.

CMS quality programs and value-based payment models do not currently recognize administrative efficiency as a measured outcome. Yet administrative burden is a primary driver of clinician burnout, which directly impacts care quality, safety, and workforce retention.

Recommendation: HHS should explore whether workforce stability metrics (turnover, vacancy rates, time-to-fill) and administrative burden measures could be incorporated into quality reporting or value-based payment adjustments, creating financial incentives for health systems to invest in operational AI that reduces burden.

Question 7: Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

Key decision-makers for operational AI adoption:

- **Chief Nursing Officers (CNOs) and VP/Directors of Nursing Operations** — These leaders feel the pain of scheduling complexity most acutely and are often initial champions.
- **VP of Operations or Chief Operating Officer** — Controls budget authority and operational priorities.

- **IT Security and Compliance** — Gatekeepers who evaluate data handling, integration risk, and vendor security posture.
- **HRIS/Workforce Management System Owners** — Often skeptical of tools that interact with "their" systems; can slow or block integration approvals.

Primary administrative hurdles:

1. **No single owner for operational AI.** Unlike clinical AI (which falls under CMO/CMIO purview), operational AI spans nursing, HR, IT, and operations. This creates confusion about who evaluates, approves, and owns these tools.
2. **Procurement processes designed for large capital expenditures.** AI tools often have SaaS pricing models with lower upfront costs but recurring fees. Procurement teams trained on capital equipment evaluation struggle to assess these models, leading to delays.
3. **Security review bottlenecks.** Enterprise security teams are overwhelmed; vendor security reviews can take 3-6 months even for low-risk operational tools.
4. **Change management resistance.** Unit managers and schedulers who have "always done it this way" can resist AI tools that change their workflows, even if the tools reduce their burden. Successful adoption requires executive sponsorship and frontline engagement simultaneously.

Question 8: Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care?

Workforce and operational data interoperability is critically underserved.

The following data types, if made more interoperable, would significantly accelerate AI adoption for clinical workforce operations:

Data Type	Current State	Impact of Interoperability
Staff schedules	Locked in proprietary scheduling systems (Kronos/UKG, API Healthcare, ShiftWizard) with limited API access	AI could optimize schedules, predict gaps, automate assignments
Credentials and licenses	Siloed in credentialing software (Symplr, MD-Staff, CredentialStream)	Real-time credential verification for compliant autonomous scheduling
Time and attendance	Fragmented across payroll and HRIS systems (ADP, Paycom, Workday)	Closed-loop scheduling → time tracking → payroll automation

Float pool / PRN availability	Often managed in spreadsheets or standalone systems	AI could match available staff to open shifts across facilities
Union and contract rules	Typically hardcoded or manually enforced	AI could enforce complex work rules automatically

Recommendation: HHS should convene stakeholders like health systems, workforce technology vendors, and AI developers, to develop voluntary interoperability standards or a common data model for workforce operations data, similar to USCDI for clinical data.

Question 9: What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care?

From our conversations with health system leaders and frontline staff, the challenges most directly tied to workforce AI include:

For patients:

- **Staff continuity.** Patients, particularly in post-acute, long-term care, and oncology settings really value seeing consistent caregivers. AI-optimized scheduling can prioritize continuity assignments.
- **Reduced wait times.** Understaffing due to last-minute call-offs creates delays. AI that rapidly fills gaps improves patient access.
- **Safety.** Fatigued, burned-out staff make more errors. Reducing administrative burden and optimizing schedules for appropriate rest improves safety.

For caregivers (clinical staff):

- **Schedule predictability and fairness.** Manual scheduling often feels arbitrary or biased. AI can apply rules consistently.
- **Reduced last-minute changes.** AI can anticipate gaps and fill them proactively rather than calling staff at the last minute.
- **Less time on administrative tasks.** Nurses and managers spend hours on scheduling, swap approvals, and credential checks. Automating this returns time to patient care.

Concern to address: Caregivers express concern about AI making decisions that affect their work lives without transparency. Any workforce AI must be explainable. Staff should understand why they were assigned a particular shift and must preserve human override capability.

III. Summary Recommendations

1. **Extend interoperability policy attention to workforce management systems** to unlock the data AI needs to optimize clinical operations.
2. **Publish clear guidance distinguishing operational AI from clinical AI and medical devices**, addressing accountability, documentation, and oversight expectations proportionate to risk.
3. **Recognize administrative burden reduction and workforce stability as quality-adjacent outcomes** that merit measurement and incentive alignment in CMS programs.
4. **Convene a public-private working group on workforce data standards** to accelerate voluntary interoperability for scheduling, credentialing, and time/attendance systems.

IV. Conclusion

AI's greatest near-term impact in healthcare may not be in diagnosis or imaging — it may be in relieving the operational and administrative burden that drives clinician burnout and workforce shortages. The barriers to adoption are not primarily technical; they are structural, procedural, and policy-driven. HHS has an opportunity to accelerate responsible adoption of operational AI by clarifying accountability, extending interoperability principles to workforce data, and recognizing administrative efficiency as a component of care quality.

ShiftOS welcomes further engagement with HHS on these issues and is available to provide additional detail or participate in stakeholder convenings.

Respectfully submitted,

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